LABORATORY REPORT

Redcliffe labs

Patient NAME	**: Mr Abhay Raina**		
DOB/Age/Gender	: 44 Y 3 M 21 D/Male	Report STATUS	: Final Report
Patient ID / UHID	: 13345293/RCL12908562	Barcode NO	: 27183240
Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: Aug 01, 2025, 08:14 AM	Report Date	: Aug 01, 2025, 02:28 PM.

NABL M(EL)T LABS
NABL-M(EL)T-00631

EPM AHC With PPBS - PAN INDIA - 2025

Complete Blood Count (CBC)

Test Description	Value(s)	Unit(s)	Reference Range
RBC Parameters			
Hemoglobin *colorimetric*	13.5	g/dL	13.0 - 17.0
RBC Count *Electrical impedance*	4.7	10^6/µl	4.5 - 5.5
PCV *Calculated*	**39.9**	%	40 - 50
MCV *Electrical Impedance*	85	fl	83 - 101
MCH *Calculated*	28.7	pg	27 - 32
MCHC *Calculated*	33.8	g/dL	31.5 - 34.5
RDW (CV) * *Calculated*	**14.3**	%	11.6 - 14.0
RDW-SD * *Calculated*	43.8	fl	35.1 - 43.9
WBC Parameters			
TLC *Electrical impedance and microscopy*	4.7	10^3/µl	4 - 10
Differential Leucocyte Count			
Neutrophils	47	%	40-80
Lymphocytes	**40.5**	%	20-40
Monocytes	6.9	%	2-10
Eosinophils	5.2	%	1-6
Basophils	0.4	%	<2
Absolute Leukocyte Counts ***Calculated***			
Neutrophils.	2.21	10^3/µl	2 - 7
Lymphocytes.	1.9	10^3/µl	1 - 3
Monocytes.	0.32	10^3/µl	0.2 - 1.0
Eosinophils.	0.24	10^3/µl	0.02 - 0.5
Basophils.	0.02	10^3/µl	0.02 - 0.5
Platelet Parameters			
Platelet Count *Electrical impedance and microscopy*	150	10^3/µl	150 - 410
Mean Platelet Volume (MPV) * *Calculated*	11.4	fL	9.3 - 12.1

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Test Description	Value(s)	Unit(s)	Reference Range
PCT * *Calculated*	**0.1**	%	0.17 - 0.32
PDW * *Calculated*	14.5	fL	8.3 - 25.0
P-LCR * *Calculated*	34.8	%	18 - 50
P-LCC * *Calculated*	**31**	10^9/L	44 - 140
Mentzer Index * *Calculated*	18.09	%	> 13
Platelet Morphology * *Microscopy*	PLATELETS ARE ADEQUATE ON SMEAR WITH PLATELET CLUMPS PRESENT.	-	-

Interpretation:
CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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Test Description	Value(s)	Unit(s)	Reference Range

Erythrocyte Sedimentation Rate (ESR)

ESR - Erythrocyte Sedimentation Rate *MODIFIED WESTERGREN*	2	mm/hr	0 - 10

Interpretation:
ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis ,systemic vasculitis, anemia, multiple myeloma , etc. Low levels are typically seen in congestive heart failure, polycythemia ,sickle cell anemia, hypo fibrinogenemia , etc.

Reference- Dacie and lewis practical hematology

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Test Description	Value(s)	Unit(s)	Reference Range

HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c) *HPLC*	5.5	%	<5.7
Estimated Average Glucose *	111.15	mg/dL	Refer Table Below

Interpretation:
Interpretation For HbA1c% As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:
1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments :
HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

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Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: Aug 01, 2025, 08:14 AM	Report Date	: Aug 01, 2025, 01:22 PM.

Test Description	Value(s)	Unit(s)	Reference Range

Blood Group ABO & Rh Typing

Blood Group	B	-	-
Rh Factor	Positive	-	-

Interpretation:
Interpreting the results of a blood group test involves understanding the ABO blood group and Rh factor results. We can interpret them as follows:

ABO Blood Grouping:
Blood Group A: Has A antigens on red blood cells and anti-B antibodies in the plasma.
Blood Group B: Has B antigens on red blood cells and anti-A antibodies in the plasma.
Blood Group AB: Has both A and B antigens on red blood cells and no anti-A or anti-B antibodies in the plasma.
Blood Group O: Has no A or B antigens on red blood cells but has both anti-A and anti-B antibodies in the plasma.

Rh Typing:
Rh-positive: Indicates the presence of the Rh antigen (D antigen) on red blood cells.
Rh-negative: Indicates the absence of the Rh antigen on red blood cells.

Interpreting your blood group involves identifying which antigens are present on your red blood cells (A, B, or both) and whether you are Rh-positive or Rh-negative. For example, if your blood group is A-positive, it means you have A antigens and Rh antigen on your red blood cells.

Knowing your blood group is important for medical purposes, such as blood transfusions, organ transplants, and during pregnancy to prevent potential complications related to blood compatibility.

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Patient ID / UHID	: 13345293/RCL12908562	Barcode NO	: 27424794
Referred BY	: Self	Sample Type	: FLUORIDE F
Sample Collected	: Aug 01, 2025, 08:14 AM	Report Date	: Aug 01, 2025, 12:41 PM.

NABL M(EL)T LABS
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Test Description	Value(s)	Unit(s)	Reference Range

Glucose Fasting

Glucose Fasting *Hexokinase*	81	mg/dL	70 - 100

Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	70 - 100
Impaired fasting glucose	101 - 125
Diabetes	≥126

Reference : American Diabetes Association

Comment :
Blood glucose determinations in commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy insulinoma, or various liver diseases.

Note
1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL or a random / 2 hour plasma glucose value of > or = 200 mg/dL with symptoms of diabetes mellitus.
2.Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.

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Referred BY	: Self	Sample Type	: Serum
Sample Collected	: Aug 01, 2025, 08:14 AM	Report Date	: Aug 01, 2025, 01:33 PM.

Blood Urea Nitrogen (Bun)

Test Description	Value(s)	Unit(s)	Reference Range
Blood Urea *Urease*	42.8	mg/dL	19 - 44.1
Bun * *Calculated*	20	mg/dL	8.9 - 20.6

Interpretation:

1. The blood urea nitrogen (BUN) test measures the amount of nitrogen in your blood that comes from the waste product urea. Urea is formed in the liver when the body breaks down proteins and is eventually eliminated by the kidneys.

2. Low BUN levels can be caused by a variety of factors. Liver disease is a primary cause, as urea is produced in the liver, and any dysfunction in this organ can lead to lower BUN levels. Malnutrition or a low protein diet can also contribute, as inadequate protein intake reduces urea production. Overhydration, resulting from excessive fluid intake, can dilute the urea concentration in the blood. Additionally, pregnancy can lower BUN levels due to increased fluid retention and changes in metabolism.

3. High BUN levels can result from various conditions. Kidney dysfunction is a primary cause, as impaired kidney function can reduce urea elimination. Dehydration, which decreases fluid volume in the body, can also concentrate urea. A high protein diet can increase urea production due to excessive protein intake. Heart failure can reduce blood flow to the kidneys, impairing their function. Gastrointestinal bleeding contributes to high BUN levels by breaking down blood proteins in the digestive tract. Certain medications, such as corticosteroids and antibiotics, can elevate BUN levels. Additionally, shock or severe stress can affect BUN levels by reducing blood flow to the kidneys.

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Test Description	Value(s)	Unit(s)	Reference Range

Creatinine

Creatinine *Photometric*	0.86	mg/dL	0.72 - 1.25
eGFR (CKD-EPI)	109.27	ml/min/1.73 sq m	Normal Or High: >= 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15

Interpretation:
Creatinine estimation is done to assess kidney function. It is not dependent on dietary factors. Normal values are obtained in kidney diseases, except in advanced renal failure and therefore its estimation is more valuable if coupled with clearance.

"eGFR test is applicable for patients aged 18 years or more."

Uric Acid

Uric Acid *Uricase*	5.8	mg/dL	3.5 - 7.2

Interpretation:
Serum uric acid levels are very labile and show day to day and seasonal variation in some people. Levels are also increased by emotional stress, total fasting and increased body weight. Serum uric acid levels are used to diagnose and monitor treatment of gout, monitor chemotherapeutic treatment of neoplasms to avoid renal urate deposition with possible renal failure.

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Liver Function Test (LFT)

Test Description	Value(s)	Unit(s)	Reference Range
Bilirubin Total *Photometric*	0.8	mg/dL	0.2 - 1.2
Bilirubin Direct * *Diazo Reaction*	0.3	mg/dL	0.0 - 0.5
Bilirubin Indirect * *Calculation (T Bil - D Bil)*	0.5	mg/dL	0.1 - 1.0
SGOT/AST *IFCC without P5P*	32	U/L	5 - 34
SGPT/ALT *IFCC without P5P*	23	U/L	0 to 55
SGOT/SGPT Ratio *	1.39	-	-
Alkaline Phosphatase *IFCC*	73.33	U/L	40 - 150
Total Protein *Biuret*	6.8	g/dL	6.4 - 8.3
Albumin *BCG*	4.62	gm/dL	3.8 - 5.0
Globulin * *Calculation (T.P - Albumin)*	**2.18**	g/dL	2.3 - 3.5
Albumin :Globulin Ratio * *Calculation (Albumin/Globulin)*	**2.12**	-	1.0 - 2.1
Gamma Glutamyl Transferase (GGT) * *Photometric*	19	U/L	12 - 64

Interpretation:
The liver filters blood, metabolizes nutrients, detoxifies harmful substances, and produces blood clotting proteins. Liver cells contain enzymes that facilitate these functions. When cells are damaged, enzymes leak into the blood, detectable through blood tests.

Key enzymes tested:

1. **AST (SGOT):** may indicate tissue injury / damage in muscles or liver.
2. **ALT (SGPT):** Primarily in the liver. Elevated ALT and AST suggest liver damage.
3. **Alkaline Phosphatase & GGT:** Linked to bile production and flow. Elevated levels may indicate bile flow issues related to the liver, gallbladder, or bile ducts.

Blood proteins, **albumin and globulin**, are essential for growth, development, and health.

1. **Low protein:** May indicate bleeding, liver disorders, malnutrition, or agammaglobulinemia.
2. **High protein (Hyperproteinemia):** Often due to dehydration or increased protein production.
3. **Low albumin:** Caused by poor diet, kidney, or liver disease.
4. **High albumin:** Usually due to severe dehydration.

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Test Description	Value(s)	Unit(s)	Reference Range
Lipid Profile			
Total Cholesterol *Enzymatic - Cholesterol Oxidase*	185	mg/dL	<200
Triglycerides *Colorimetric - Lip/Glycerol Kinase*	49	mg/dL	<150
HDL Cholesterol *Accelerator Selective Detergent*	64	mg/dL	>40
Non HDL Cholesterol * *Calculated*	121	mg/dL	<130
LDL Cholesterol * *Calculated*	**111.2**	mg/dL	<100
V.L.D.L Cholesterol * *Calculated*	9.8	mg/dL	< 30
Chol/HDL Ratio * *Calculated*	**2.89**	Ratio	3.5 - 5.0
HDL/ LDL Ratio * *Calculated*	0.58	Ratio	0.5 - 3.0
LDL/HDL Ratio * *Calculated*	1.74	Ratio	-

Interpretation:
Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays.

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High
<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence

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Test Description	Value(s)	Unit(s)	Reference Range

High Risk	of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >/= 50 mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age >/=45 years in Males & >/= 55 years in Females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

*** After an adequate non-pharmacological intervention for at least 3 months.**

References : Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology,2022,20,134-155.

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Test Description	Value(s)	Unit(s)	Reference Range

Calcium

Calcium Serum *Arsenazo III*	9.3	mg/dL	8.4 - 10.2

Interpretation:
Elevated calcium value are associated with hyperparathyrodism, multiple myeloma, neoplasms of bone and parathyroid & conditions of rapid demineralization, tetany & occasionally with nephrosis & pancreatitis. Severe nephritis & uremia may cause either elevated or lowered calcium values. Decreased values of calcium are noted in hypoparathyroidism, vitamin D deficiency, renal insufficiency, hypoproteinemia, malabsorption syndrome, severe pancreatitis with pancreatic necrosis and pseudo-hypoparathyroidism.

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Test Description	Value(s)	Unit(s)	Reference Range

Vitamin D 25 Hydroxy

Vitamin D 25 - Hydroxy *CMIA*	57.4	ng/mL	Deficiency : <30 ng/mL

Interpretation:
25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

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Test Description	Value(s)	Unit(s)	Reference Range
Thyroid Profile Total			
Triiodothyronine (T3) *CMIA*	62.4	ng/dL	35 - 193
Total Thyroxine (T4) *CMIA*	5.1	µg/dL	4.87 - 11.2
Thyroid Stimulating Hormone (Ultrasensitive) *CMIA*	1.7841	mIU/L	0.35 - 4.94

Interpretation:

Pregnancy	Refrence Range TSH
1st Trimester	0 .1 - 2.5
2nd Trimester	0.2 - 3.0
3rd Trimester	0.3 - 3.0

Clinical Use:
1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

Increased TSH: Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance
Decreased TSH: Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency

Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

1. Primary Hypothyroidism: High TSH levels.
2. Secondary/Tertiary Hypothyroidism: Low TSH levels.
3. Euthyroid Sick Syndrome: Abnormal thyroid test results due to non-thyroidal illnesses (NTI).

TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

TSH	T4	T3	Interpretation
High	Normal	Normal	Mild (subclinical) hypothyroidism
High	Low	Low Or Nomral	Hypothyroidism
Low	Normal	Normal	Mild (subclinical) hyperthyroidism
Low	High Or Normal	High Or Normal	Hyperthyroidism
Low	Low Or Normal	Low Or Normal	Nonthyroidal illness; pituitary (secondary) hypothyroidism
Normal	High	High	Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function)

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Test Description	Value(s)	Unit(s)	Reference Range

Phosphorus

Phosphorus *Photometric*	3.6	mg/dL	2.3 - 4.7

Interpretation:
Increased levels are found in acute mycocardial infarction (AMI), AMI with congestive heart failure, cirrhosis, obstructive jaundice, hemolytic anemia, various malignancies, skeletal muscle injury, electric and thermal burns.

Electrolytes (Na/K/Cl)

Sodium *Potentiometric*	137	mmol/L	136 - 145
Potassium *Potentiometric*	3.8	mmol/L	3.5 - 5.1
Chloride *Potentiometric*	103	mmol/L	98 - 107

Interpretation:
An electrolyte panel, which typically includes measurements of sodium (Na), potassium (K), and chloride (Cl) levels, is a common blood test that provides information about your body's electrolyte balance.

1. Sodium (Na):
1. Sodium is crucial in maintaining fluid balance in your body and is essential for nerve function and muscle contraction.
2. High sodium levels (hypernatremia) or low sodium levels (hyponatremia) can indicate various health conditions, including dehydration, kidney problems, or hormonal imbalances.

2. Potassium (K):
1. Potassium is vital for proper muscle function, including the heart muscle and nerve function, as well as maintaining fluid and electrolyte balance.
2. Abnormal potassium levels (hyperkalemia or hypokalemia) can indicate kidney dysfunction, dehydration, certain medications, or other underlying health issues.

3. Chloride (Cl):
1. Chloride works closely with sodium and potassium to maintain fluid balance and proper pH levels in the body.
2. Abnormal chloride levels may occur alongside imbalances in sodium and potassium and can indicate dehydration, kidney problems, respiratory issues, or metabolic disorders.

Healthcare providers typically order an electrolyte panel to evaluate and monitor electrolyte imbalances, diagnose certain medical conditions, assess kidney function, or monitor treatment effectiveness. It's usually part of routine blood testing, especially in cases of dehydration, kidney disease, heart conditions, or electrolyte disorders.

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

Dr. Kamal Jadav
MD (Pathology)
Consultant Pathologist

Patient NAME	**: Mr Abhay Raina**		
DOB/Age/Gender	: 44 Y 3 M 21 D/Male	Report STATUS	: Final Report
Patient ID / UHID	: 13345293/RCL12908562	Barcode NO	: 26934614
Referred BY	: Self	Sample Type	: Serum
Sample Collected	: Aug 01, 2025, 08:14 AM	Report Date	: Aug 01, 2025, 01:33 PM.

Test Description	Value(s)	Unit(s)	Reference Range

Apolipoprotein A1 (Apo A)

Apolipoprotein A-1 *Tina-quant*	190	mg/dL	104 - 202

Interpretaion:
Apolipoprotein A1 is one of the apoproteins of high density lipoproteins (HDL) which is inversely related to the risk of CAD. Individuals with Tangier disease have < 1% of normal Apo A1. Levels <90 mg/dL indicate increased risk of Atherosclerotic disease.

Apolipoprotein B (Apo B)

Apolipoprotein B *Tina-quant*	78	mg/dL	66 - 144

Interpretaion:
Apolipoprotein B is a more powerful independent predictor of Coronary Heart Disease (CAD) than LDL Cholesterol. It is useful in assessing the risk of CAD and to classify Hyperlipidemias. Apolipoprotein studies help in monitoring coronary bypass surgery patients with regard to risk and severity of re -stenosis. They are also useful in assessing risk of re-infarction in patients of Myocardial infarction.

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Referred BY	: Self	Sample Type	: Serum
Sample Collected	: Aug 01, 2025, 08:14 AM	Report Date	: Aug 01, 2025, 12:50 PM.

Test Description	Value(s)	Unit(s)	Reference Range

Hepatitis B Surface Antigen (HBsAg), Rapid Card

HEPATITIS B SURFACE ANTIGEN (HBsAg) *Qualitative immunoassay,rapid card*	NON REACTIVE		NON REACTIVE

Interpretation:

RESULTS	REMARKS
Reactive	The sample is Reactive for HBsAg
Non Reactive	The sample is Non Reactive for HBsAg

Note

1.**This is only a Screening test.** All reactive results should be confirmed by confirmatory test. Therefore for a definitive diagnosis, the patient's clinical history ,symptomatalogy as well as serological data, should be considered. The results should be reported only after complying with above procedure.

2. Additional follow up testing using available clinical methods (along with repeat HBsAg rapid card test) is required, if the test is Non reactive with persisting clinical symptoms

3. False positive results may be observed in patients receiving mouse monoclonal antibodies, on heparin therapy, on biotin supplements for diagnosis or therapy, presence of heterophilic antibodies in serum or after HBV vaccination for transient period of time.

4. False negative reaction may be due to processing of sample collected early in the course of disease or presence of mutant forms of HBsAg.

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Consultant Pathologist

LABORATORY REPORT

Redcliffe labs

Patient NAME	**: Mr Abhay Raina**		
DOB/Age/Gender	: 44 Y 3 M 21 D/Male	Report STATUS	: Final Report
Patient ID / UHID	: 13345293/RCL12908562	Barcode NO	: 27308965
Referred BY	: Self	Sample Type	: Spot Urine
Sample Collected	: Aug 01, 2025, 08:14 AM	Report Date	: Aug 01, 2025, 12:22 PM.

NABL M(EL)T LABS
NABL-M(EL)T-00631

Urine Routine and Microscopic Examination

Test Description	Value(s)	Unit(s)	Reference Range
Physical Examination			
Volume *	20	ml	-
Colour *	Pale yellow	-	Pale yellow
Transparency *	Clear	-	Clear
Deposit *	Absent	-	Absent
Chemical Examination			
Reaction (pH) *Double Indicator*	6.0	-	4.5 - 8.0
Specific Gravity *Ion Exchange*	1.015	-	1.010 - 1.030
Urine Glucose (sugar) *Oxidase / Peroxidase*	Negative	-	Negative
Urine Protein (Albumin) *Acid / Base Colour Excahnge*	Negative	-	Negative
Urine Ketones (Acetone) *Legals Test*	Negative	-	Negative
Blood *Peroxidase Hemoglobin*	Negative	-	Negative
Leucocyte esterase *Enzymatic Reaction*	Negative	-	Negative
Bilirubin Urine *Coupling Reaction*	Negative	-	Negative
Nitrite *Griless Test*	Negative	-	Negative
Urobilinogen *Ehrlichs Test*	Normal	-	Normal
Microscopic Examination			
Pus Cells (WBCs) *	2-4	/hpf	0 - 5
Epithelial Cells *	1-2	/hpf	0 - 4
Red blood Cells *	Absent	/hpf	Absent
Crystals *	Absent	-	Absent
Cast *	Absent	-	Absent
Yeast Cells *	Absent	-	Absent
Amorphous deposits *	Absent	-	Absent
Bacteria *	Absent	-	Absent
Protozoa *	Absent	-	Absent

Interpretation:
URINALYSIS- Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders.

Protein: Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever

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Consultant Pathologist

Patient NAME	**: Mr Abhay Raina**		
DOB/Age/Gender	: 44 Y 3 M 21 D/Male	Report STATUS	: Final Report
Patient ID / UHID	: 13345293/RCL12908562	Barcode NO	: 27308965
Referred BY	: Self	Sample Type	: Spot Urine
Sample Collected	: Aug 01, 2025, 08:14 AM	Report Date	: Aug 01, 2025, 12:22 PM.

NABL M(EL)T LABS
NABL-M(EL)T-00631

Test Description	Value(s)	Unit(s)	Reference Range

Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications.

Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise.

Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders.

Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection.

Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection.

pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine.

Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus.

Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine.

Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.

*** End Of Report ***

Pending Report To Follow - Stool R/M, Glu-PP

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Consultant Pathologist

LABORATORY REPORT

Redcliffe labs

Bio-Rad CDM System
VII Inst. #1. SN- 17012

PATIENT REPORT
V2TURBO_A1c_2.0

Patient Data

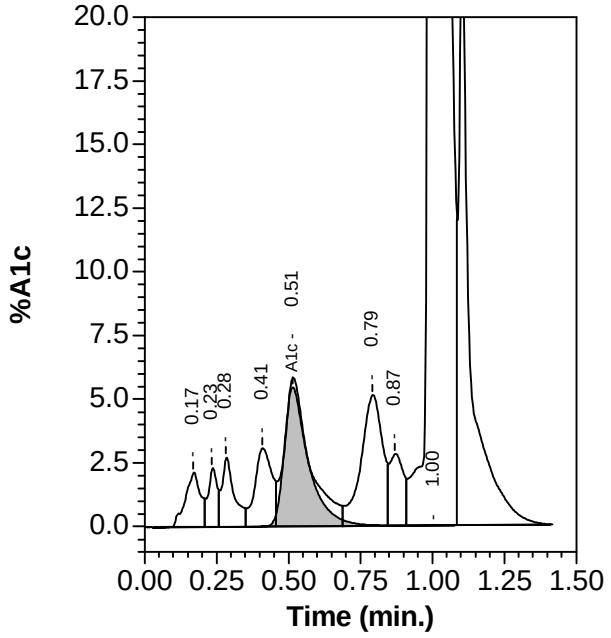
Sample ID: 27183240
Patient ID:
Name:
Physician:
Sex:
DOB:

Comments:

Analysis Data

Analysis Performed: 01/08/2025 12:33:17
Injection Number: 1522
Run Number: 106
Rack ID: 0010
Tube Number: 1
Report Generated: 01/08/2025 12:43:27
Operator ID:

Peak Name	NGSP %	Area %	Retention Time (min)	Peak Area
A1a	---	1.1	0.167	13793
A1b	---	0.7	0.232	8834
F	---	1.1	0.281	14102
LA1c	---	1.7	0.409	22476
A1c	5.5	---	0.514	56229
P3	---	3.6	0.789	46572
P4	---	1.3	0.868	16537
Ao	---	86.3	1.005	1121987

Total Area: 1,300,532

HbA1c (NGSP) = 5.5 %